

{org.name} ABN {org.abn}

Administering Medication Checklist & Medication Chart

Client Details		
Name:		Insert Image:
Phone Number:		
Date of Birth:	___ / ___ / ___	
Confirmed allergens:		
Family/Emergency contact name(s):		
Work Phone:		
Home Phone:		
Mobile Phone:		
Prescriber Details		
Name of Prescriber		
PBS Prescriber Number		
Contact Number		
Address of Doctor's health service organisation		
Date of Prescribing		
Prescriber signature		
Date		

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Pharmacy/Phone Number	
Date form last updated	___ / ___ / ___

1. As required PRN Medicines

Start Date	Medicine (print generic name)/form	Date							
Route	Dose and hourly frequency	PRN	Time						
Indication	Max PRN dose/24hr	Dose							
SAC/AAN	Pharmacy	Route							
Prescriber Signature	Print Name	Sign							
Start Date	Medicine (print generic name)/form	Date							
Route	Dose and hourly frequency	PRN	Time						
Indication	Max PRN dose/24hr	Dose							
SAC/AAN	Pharmacy	Route							
Prescriber Signature	Print Name	Sign							

Start Date	Medicine (print generic name)/form		Date						
Route	Dose and hourly frequency PRN		Time						
Indication		Max PRN dose/24hr	Dose						
SAC/AAN		Pharmacy	Route						
Prescriber Signature			Print Name	Sign					
Start Date	Medicine (print generic name)/form		Date						
Route	Dose and hourly frequency PRN		Time						
Indication		Max PRN dose/24hr	Dose						
SAC/AAN		Pharmacy	Route						
Prescriber Signature			Print Name	Sign					

2. As required PRN Medicines prescribing instructions

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The prescriber must write the:

- Start date of prescription
- Generic medicine name (see under 'Regular Medicines' for further comments)
- Route of administration
- Dose AND hourly frequency. "PRN" (pre-printed) alone is not sufficient
- Indication and maximum daily dose (i.e. maximum dose in 24 hours) For example: Maximum of 4g paracetamol in 24 hours

Prescriber's signature, printed name and contact details The 'Max PRN dose/24 hr' prompt indicates the total amount of the medication which may be administered for PRN doses only for that medication.

The maximum daily dosage should not be exceeded for that PRN medicine. Prescribers should exercise caution when prescribing PRN medications, and check the regular medication section for possible duplicated orders. Where appropriate, pharmacists should annotate on the medication chart where a medication is prescribed in both the regular dose and PRN sections.

3. As required PRN Medicines administration instructions

For each medication administration, document the:

- Date
- Time
- Dose administered
- Route
- Initial of person administering dose
- The person administering each dose is responsible for:
 - checking that the maximum daily dosage will not be exceeded
 - checking the timing of the previous dose (includes both regular and PRN dose)

An example of how prescribers and administrators are to complete the chart is set out below:

Start Date 1/2/2021	Medicine (print generic name)/form Paracetamol	Date	2/2						
Route PO	Dose and hourly frequency 1g 4 hourly	PRN	Time 1400						
Indication Pain	Max PRN dose/24hr 4g	Dose	1g						
SAC/AAN N/A	Pharmacy Imprest	Route	PO						
Prescriber Signature <i>Mike Smith</i>	Print Name M.Smith 8948	Sign	MS						

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4. Regular Medicines

Brand substitution not permitted ☐

Date and month →									
Start Date	Warfarin	Marevan / Coumadin (circle brand)	INR Result						
Route	Prescriber to enter individual doses	Target INR Range	Dose	mg	mg	mg	mg	mg	mg
Indication		Pharmacy	Prescriber						
Prescriber signature			Initial 1 18:00						
			Initial 2						

Date and month →								
Prescriber to enter administration times →								
Start Date	Medicine (print generic name)/form Indicate if slow release							

Route	Dose and hourly frequency and now enter times→						
Indication	Pharmacy						
Prescriber signature	Print Name						

Date and month →						
Prescriber to enter administration times↴						
Start Date	Medicine (print generic name)/form Indicate if slow release					
Route	Dose and hourly frequency and now enter times→					
Indication	Pharmacy					
Prescriber signature	Print Name					

Date and month →						
Prescriber to enter administration times↴						
Start	Medicine (print generic name)/form					

Date	Indicate if slow release						
Route	Dose and hourly frequency and now enter times→						
Indication	Pharmacy						
Prescriber signature	Print Name						

Date and month →							
Prescriber to enter administration times↗							
Start Date	Medicine (print generic name)/form Indicate if slow release						
Route	Dose and hourly frequency and now enter times→						

Indication	Pharmacy						
Prescriber signature	Print Name						

5. Regular Medicines prescribing instructions

These are the minimum requirements that need to be documented by the prescriber for a medication order to be valid:

Start Date	The prescriber should enter the current date if administration of the medication is intended on this date or a date in the future when the prescriber wants the first administration of the medication. If transcribing/re-writing charts, the prescriber must enter the date of transcription. It is not the date that the medication was originally ordered. It may be useful for some medications (e.g. antibiotics) to include the date of first prescribing somewhere on the administration panel.
Generic medicine name	Medications must be ordered preferably by their generic name (active ingredient). Medications must be prescribed by their full name. Medication names must never be abbreviated. For example: ISMN – isosorbide mononitrate GTN – glyceryl trinitrate EPO – epoetin Medications must not be prescribed by chemical names/symbols. For example: KCl – potassium chloride Local policy or guidelines may allow the use of trade names (brand names) of combination products (e.g. Seretide®). Medication orders for insulin preparations (irrespective of whether it is a single or multiple ingredient product) should preferably be written in trade name – to minimise confusion between available products. It is also recommended to include the form of insulin device being used (i.e. NovoRapid Flexpen® 10 Units tds). The Australian Approved Name is the official terminology as per the Therapeutic Goods Administration (TGA) website A separate order is required for each medication. To reduce the chance of error, in circumstances where a combination product is unavailable as a single product, but available in separate components, the medication order must be rewritten to reflect what is being administered. For example: Clopidogrel/Aspirin 75/100mg 1 tablet mane – rewritten as separate orders for:

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	o Clopidogrel 75mg mane, AND o Aspirin 100 mg mane
Route	Generally, each order is for one route only. Health services should be aware of risks associated with medication orders with multiple routes of administration, ensuring there is adequate documentation of the actual route administered and whether the dosing between routes is bioequivalent. Local policy or guidelines may allow multiple routes to be ordered together (e.g. where the dose required for different routes are the same – for example paracetamol 1g [PO or IV] qid). A health service-specific list of exceptions to the general rule should be determined in conjunction with the health service's drug and therapeutics committee or equivalent and appropriate risk mitigation strategies put in place. Note: Writing a 'regular dose' order for multiple routes of medication does not allow the documentation of routes used to administer the dose.
Dose	Only metric and Arabic numerals must be used. Roman numerals (e.g. i, ii, iii, iv, etc.) must be avoided. Always use a zero (0) before a decimal point e.g. 0.5g otherwise the decimal point may be missed. However, if possible, it is preferable to state the dose in whole numbers, not decimals. For example, write 500mg instead of 0.5g, or write 125 micrograms instead of 0.125mg. Never use a trialling zero ('.0') as it might be misread if the decimal point is missed e.g. 1.0 misread as 10. Do not use 'U' or 'IU' for units because the 'U' may be misread as zero. Always write units in full. Note: In the case of liquid medications, the strength and dose in milligrams or micrograms must always be specified (not in millilitres). For example, for a medication order for 10mg of morphine liquid would read: Correct: Morphine mixture (10mg/mL) – Give 10 mg every 8 hours when required Wrong: Morphine mixture (10mg/ml) – Give 1mL every 8 hours when required The amount in 'mL' can be annotated on the chart in addition to the strength to clarify the amount to be administered. The clinical/ward pharmacist should clarify the order, where the strength of the medication supplied is different from that which has been ordered. For example: 10mg dose: Pharmacist will write "2 x 5mg tablet" 25mg dose: Pharmacist will write "Half x 50mg tablet"
Frequency and Administration time(s)	The prescriber must enter both the frequency AND administration time(s) for the medication. If not entered, the dose may not be administered by nursing staff. Administration times should be entered using the 24 hour clock, a universal standard. Medications should be administered according to the recommended administration times unless they must be given at specific times (e.g. some antibiotics, with/before food, Parkinson's disease medications) or, as in the case of children with variable meal

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	and sleep schedules, a specific schedule is required. If necessary, a pharmacist or nurse should clarify the administration time to correctly administer the medication (e.g. in relation to food) and annotate the chart to indicate that this has occurred. In the situation where a medication is prescribed more than six times per day (e.g. eye drops requiring administration every 2 hours), the medication order should be written over two consecutive boxes to allow more than 6 dosage times to be entered.
Indication	This is critical information as it allows the order to be reviewed in the context of why it was prescribed, therefore reducing the risk of misinterpretation of the order (e.g. look alike sound alike [LASA] medications, incorrect doses, medications which have different doses for different indications) Recording the indication for each medication helps the health care team select the right medication.
Prescriber signature	The signature of the prescriber must be documented to complete each medication order. For each signature (prescriber), their name must be printed at least once on that medication chart, preferably with a contact number to allow clarification, if necessary.
Slow release	The “Indicate if Slow Release” text is included as a prompt to prescribers to consider whether or not the standard release form of the medication is required. Prescribers should write “Slow release” to indicate a sustained or modified release form of an oral medication (e.g. verapamil SR, diltiazem CD, metformin XR, tramadol SR). If not indicated, then it is assumed that the standard release form is to be administered. If not indicated, nursing staff may want to contact the prescriber to seek clarification of which form should be administered to the Client. Further explanation is included in the margin of the medication chart. This box should also be ticked for medications which are enteric coated, as this is classified as a modified release form (releases the medication slowly within the intestine). For more information, refer to: Department of Health “Don’t Rush to Crush” poster.
Pharmacy	This section is for use by the pharmacist.

6. Nurse Initiated Medicine

Start Date	Medicine (print generic name)/form	Date							
Route	Dose and hourly frequency	Time							
RN Signature	Print Name	Sign							
Start Date	Medicine (print generic name)/form	Date							

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Route	Dose and hourly frequency	Time							
RN Signature	Print Name	Sign							
Start Date	Medicine (print generic name)/form	Date							
Route	Dose and hourly frequency	Time							
RN Signature	Print Name	Sign							

7. Administration

Every person competent and qualified in medication administration has a responsibility to ensure they can clearly read and understand the order before administering any medications. For all incomplete or unclear (include illegible) orders, the prescriber must be contacted to clarify.

Assumptions should never be made about the prescriber's intent.

Every medication chart must have the patient's identification details completed. If the patient identification details are not completed, there is no confirmation that the medication was prescribed for the correct patient.

The first person administering medications on any given day must write the date of that day in the appropriate box at the very top of each column ('Date and Month'). This column is then for medications administered on this day/date only.

The right patient

- Does the order match the patient? Ask the patient for their first and last name (3 identifiers are required to be confirmed, if possible).
- Does the label on the dispensed medication match the patient?

The right medication

- Does the medication match the order? Be vigilant with look-alike sound alike medications.
- Is it the correct medication for that particular condition?

The right dose

- Does the strength and dosage match the order?
- Does the dose require either half, whole, or multiple tablets/capsules, etc.?
- Is it safe for the patient?

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The right time

- Does the administration time match the order? Consider special precautions for timing of dose (e.g. with food).
- Does the frequency match the order?
- Ensure specified time interval has passed before administering PRN medication

The right route

- Does the route match the order?
- Is this route suitable for the patient?
- Can this preparation be crushed (tablet), opened (capsule), or mixed in other substances?
- Have old transdermal patches been removed prior to applying a new patch?

The right documentation

- Ensure that the order is valid (dated and signed by the prescriber)
- Document immediately after the medication has been administered
- If dose is not administered, document the reason for this on the medication chart

8. Reasons for not administering

Reasons for not administering

- Absent - A
- Fasting - F
- On leave - L
- Not available (obtain supply or contact prescriber) - N
- Refused - notify subscriber - R
- Self administered - S
- Vomiting - V
- Withheld - enter reason in clinical record - W

When it is not possible to administer the prescribed medication, the reason for not administering must be recorded by entering the appropriate code and circling this code. By circling the code, it will not accidentally be misread as someone's initials.

If a patient refuses a dose, the prescriber must be notified.

If a medication or dose is withheld, the reason must be documented in the patient's medical notes.

If the medication is not available on the ward, it is the nurse's responsibility to notify the pharmacy and/or obtain supply, or to contact the prescriber (or another doctor from the treating team) to advise that the medication ordered is not available.

9. Escalation

Any issues or discrepancies please contact {org.name} on {org.phone} and the Prescriber specified above.

If an emergency, use the following process:

1. USE NEAREST PHONE TO SITE OF EMERGENCY CALL 000 AND ASK FOR AMBULANCE
2. STATE ADDRESS
3. CALL BACK NUMBER – {org.phone} or an alternate phone
4. Number of Patients

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- 5. Symptoms eg Chest Pain
- 6. ADVISE – State of consciousness
- 7. Patient’s Name and Date of Birth

BE PREPARED TO MEET THE AMBULANCE

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